NCCP 00714 · clinician draft

# Nivolumab, ipilimumab, pemetrexed and cisplatin

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Cycle 2 onwards Day 1 · nivolumab + ipilimumab every 42 days

**Active medicines:** nivolumab + ipilimumab

**Emetogenic risk:** low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally | Day 1 before treatment, if following the NCIS low-risk default<br>Individualise/local policy. No automatic antiemetic corticosteroid course. |
| Metoclopramide | 10 mg orally three times daily as needed | Subsequent days as prescribed<br>Specify duration and check contraindications/interactions. |

### Additional phase-specific support

- Ipilimumab is low risk and nivolumab minimal; this combined visit is low risk.
- No further pemetrexed or platinum: stop/review induction antiemetics and rash steroid; complete the folate tail only.
- Continue immune symptom education, bloods each treatment and glucose/TFT each 42-day cycle.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00714 v4 · pNCCP 00714 supportive care; NCIS V8 Table 1 p4 · NCIS low-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required